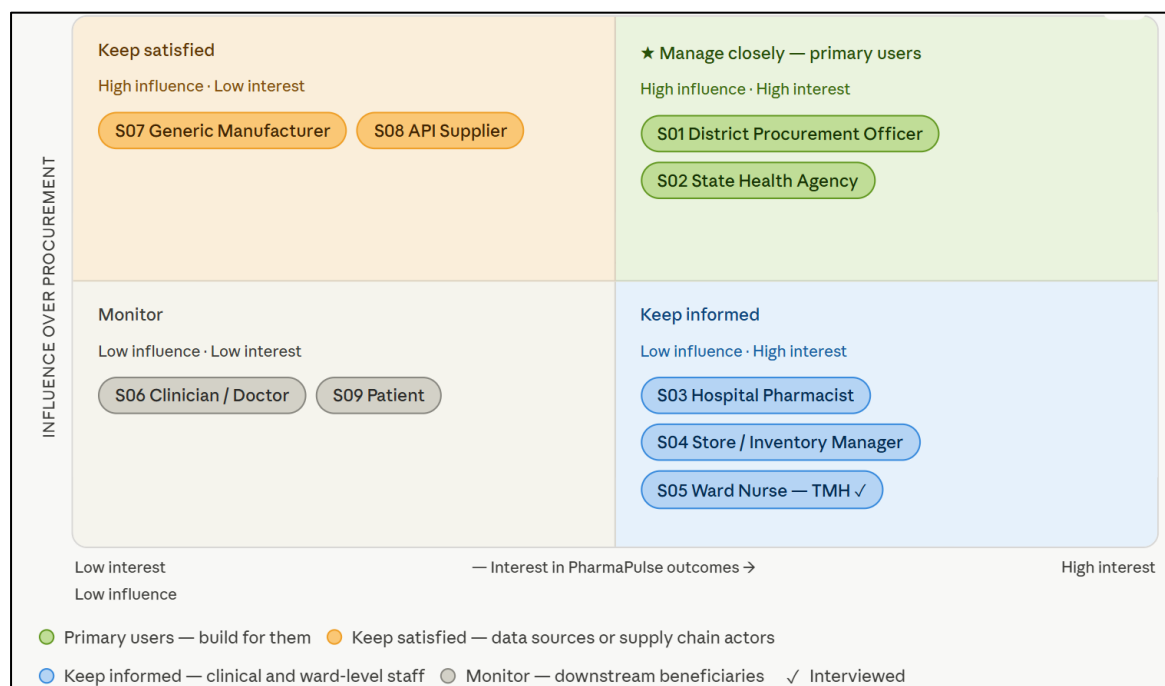


2. Process Mapping

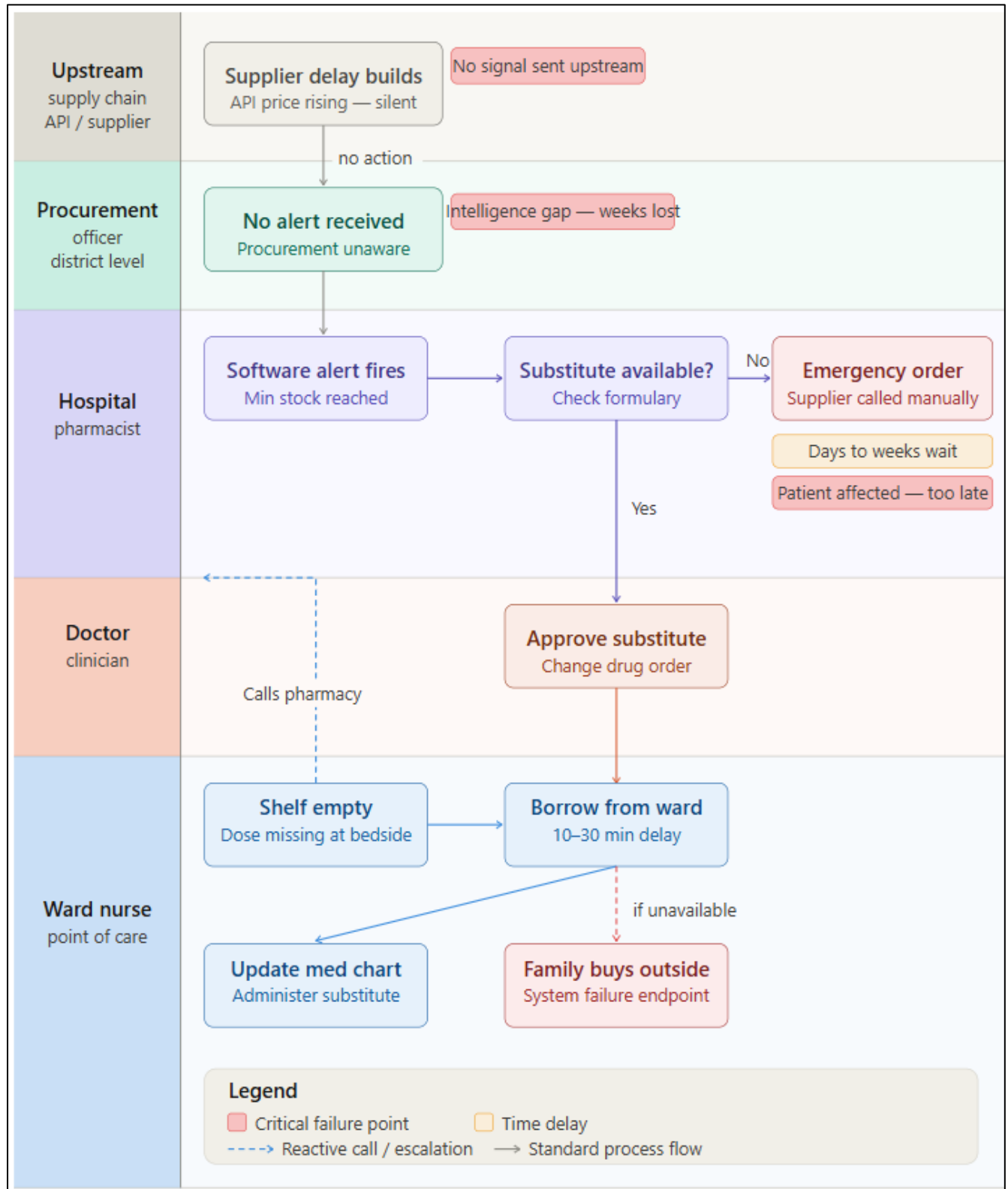
Stakeholder	Priority	Key Value from Pharma-Pulse	Current Challenge
District Procurement Officer	High	Early shortage prediction for procurement planning	Reactive purchasing decisions
State Health Agency Officer	Critical	Portfolio-wide risk visibility	Tender gaps and lack of forecasting
Hospital Pharmacist	Medium	Advance alerts for substitution planning	Late notification of shortages
Store / Inventory Manager	Medium	Improved stock visibility and reconciliation	Physical vs. system stock mismatch
Treating Clinician / Doctor	Low	Reduced treatment disruption	Forced protocol changes
Generic Drug Manufacturer	Medium	Better demand predictability	No downstream demand visibility
API Supplier / Manufacturer	Low	Upstream demand signal awareness	Limited market visibility
Patient / Beneficiary	Indirect	Improved medicine availability	Delays and medication shortages

INFLUENCE-INTEREST MATRIX



AS-IS Flow:

The following swim-lane process map documents the current drug shortage response workflow across five operational actors at the ward and hospital level. The map was constructed from primary research and identifies three critical failure points in the process.



Gap Analysis

Gap	Current State	Required State	PharmaPulse Intervention
Upstream signal visibility	Zero — no one monitors API prices or supplier health	8-12 week early warning on supply disruption	Upstream data layer — API prices, China export volume, GeM tender patterns
Procurement intelligence	Reactive — acts on empty shelf	Proactive — acts on risk score	Weekly risk dashboard for procurement officers
Cross-institution awareness	Siloed — each hospital manages alone	Connected — state-level view	State-level shortage heatmap
Decision support	Manual substitution under pressure	Pre-planned buffer stock and alternate suppliers	Recommendation engine with supplier reliability scores
Patient protection	Family sometimes buys outside	Drug always available	Shortage prevented before it reaches the ward